

OREGON STATE HOSPITAL

POLICY

SECTION 6: Patient Care **POLICY: 6.056**

SUBJECT: Suicide Risk Screening and Assessment

POINT PERSON: Chief of Psychiatry

APPROVED: Sara Walker, MD  **DATE: DECEMBER 17, 2024**
Interim Superintendent

SELECT ONE: ☐ New policy ☐ Minor/technical revision of existing policy
☐ Reaffirmation of existing policy ☒ Major revision of existing policy

I. PURPOSE AND APPLICABILITY

- A. Oregon State Hospital (OSH) recognizes suicide risk assessment is a best practice towards reducing the risk of completed suicide. OSH also recognizes that there are no clear, validated predictive models or risk stratification definitions for suicide prevention. In accordance with clinical best practices, suicide risk screenings and assessments must be completed per this policy and related procedures.
- B. This policy applies to all staff.

II. POLICY

- A. Staff designated in Procedures A must perform routine and non-routine suicide risk screenings and assessments for patients according to the schedules and criteria described in the procedures.
- B. The scope of the suicide risk assessment must be based on the patient's known risk factors, current clinical presentation, and other factors the clinician deems relevant to the assessment.
- C. As indicated by the suicide risk assessment, the interdisciplinary treatment team (IDT) must develop interventions to mitigate the patient's risk for suicide.
- D. Any staff who becomes aware of the clinical need for a suicide risk screening or assessment must immediately notify the Lead Nurse on duty. If the Lead Nurse is a Licensed Practical Nurse (LPN), the LPN must immediately notify the covering Registered Nurse (RN).

- E. A suicide risk screening or assessment must be performed when there is an acute clinical indication to do so. Examples of clinical indications that may necessitate unplanned or more frequent suicide risk screening or assessments include, but are not limited to:
 - 1. suicidal or self-injurious statements or behaviors;
 - 2. the presence of new-onset or increased mood lability, anxiety, or agitation;
 - 3. a change in frequency, severity, or content of command hallucinations or new command hallucinations;
 - 4. worsening symptoms of an eating disorder;
 - 5. refusal of recommended medical treatments for serious medical conditions;
 - 6. any other abrupt change in clinical presentation;
 - 7. new psychosocial stressors, particularly in the context of posttraumatic stress disorder (PTSD), a personality disorder, or mood disorder; or
 - 8. the emergence or change in any of a patient's signature risk signs related to suicidal thoughts, feelings, or behaviors.
- F. Registered Nurses, Psychologists, Licensed Professional Counselors (LPC), Licensed Marriage and Family Therapists (LMFT), Psychiatrists, and Psychiatric Mental Health Nurse Practitioners (PMHNP) must complete annual training and competence assessment, including a review of the components of a suicide risk assessment, current best practices, and the provisions of this policy.
- G. OSH follows all applicable regulations, including federal and state statutes and rules; Oregon Department of Administrative Services (DAS), Shared Services, and Oregon Health Authority (OHA) policies; and relevant accreditation standards. Such regulations supersede the provisions of this policy unless this policy is more restrictive.
- H. Staff who fail to comply with this policy or related policy attachments or protocols may be subject to disciplinary action, up to and including dismissal.

III. DEFINITIONS

- A. "High risk" means that risk factors for suicide in either the short-term or lifetime significantly outweigh the presence of protective factors.
- B. "Lifetime risk" means the risk for suicide across the lifespan and does not include the risk for suicide in the near future.

- C. “Low risk” means that risk factors for suicide are low or moderate, but protective factors substantially mitigate this risk in either the short-term or lifetime.
- D. “Moderate risk” means that risk factors for suicide are elevated but protective factors mitigate this risk either in the short-term or lifetime risk.
- E. “Screening” means a procedure in which a standardized instrument or protocol is used to identify individuals who may be at risk for suicide.
- F. “Short-term risk” is defined as the risk for suicide in the near future.
- G. “Signature risk sign” means an atypical symptom or behavior that is an early and reliable signal of impending relapse deemed to be especially relevant to the particular patient’s functioning, responsivity to treatment, and likelihood of adverse outcomes.
- H. “Staff” includes employees, volunteers, trainees, interns, contractors, vendors, and other state employees assigned to work at Oregon State Hospital (OSH).
- I. “Suicide risk assessment” means a comprehensive evaluation done by a psychologist, LPC, LMFT, psychiatrist, or PMHNP to estimate suicide risk, estimate the immediate danger to the patient, and decide on a course of treatment.

IV. PROCEDURES

Procedures A Suicide Risk Assessments

V. ATTACHMENTS

Attachment A Suicide Risk Assessment Flowchart

VI. RELATED OSH POLICIES AND PROTOCOLS

6.010 Enhanced Supervision

6.011 Treatment Care Planning

6.013 Discharge and Conditional Release Planning

6.042 Short-Term Assessment of Risk and Treatability (START)

6.045 Clinical Documentation

6.058 Admissions

7.005 Patient Rights

8.044 Contraband and Prohibited Items

Medical Department Protocol 1.006 "Psychiatry Admission Process and Documentation"

Medical Department Protocol 1.012 "Psychiatry Discharge Process and Documentation"

Nursing Protocol 2.005 "Admission of Patient"

Nursing Protocol 2.027 "Discharge of Patient"

Nursing Protocol 2.160 "Transfer – Unit to Unit and Between Salem and Junction City"

Nursing Protocol 2.230 "Suicide Risk Screening"

Psychology Protocol 4.028 "Suicide Risk Assessment"

VII. REFERENCES

42 CFR § 482.13(c)2.

Joint Commission Resources, Inc. (2024). *The joint commission comprehensive accreditation manual for behavioral healthcare and human services*, NPSG.15.01.01.Oakbrook Terrace, IL: Author.

Joint Commission Resources, Inc. (2024). *The joint commission comprehensive accreditation manual for hospitals*, NPSG.15.01.01.Oakbrook Terrace, IL: Author.